



Peptide health, done properly.

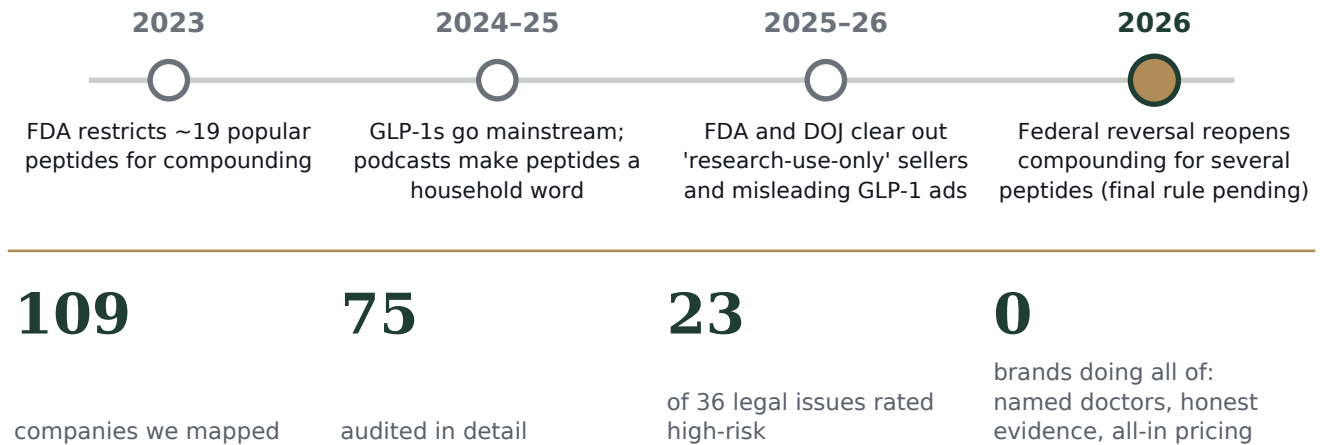
A short brief for Scott Disick
on a proposed brand partnership

Six pages. What the opportunity is, what the company would be, what your role could look like, and what we'd need to decide together.

This is a proposal for your review only. Nothing here is agreed, and nothing implies your involvement or endorsement. The full 43-page plan, research files and financial model sit behind it.

Peptides went mainstream. Trust didn't.

Between 2023 and 2026 peptides moved from gym-bro forums to dinner-party conversation, pulled along by GLP-1s, podcasts and a wave of press about the 'peptide boom'. But the way people actually buy them is a mess: sketchy 'research-use-only' websites, budget hormone clinics, and a few giant telehealth brands adding peptides late. Regulators have spent the last eighteen months clearing out the bottom of the market. Nobody has built the trustworthy front door.



Why this matters for a brand

- **The demand is real and growing.** Search interest in peptides, BPC-157 and sermorelin has climbed for three years; roughly one in eight US adults says they've used a GLP-1 drug.
- **The rules just moved in our favor.** In 2026 the federal government reversed course and began reopening prescription compounding for several popular peptides. It is not final yet, which is exactly why a compliance-first brand wins: it can move the moment the rule lands.
- **The competition is either cheap or clinical.** Budget clinics feel like steroid shops; diagnostics platforms feel like a lab report. Neither feels premium, calm, or honest about what's proven and what isn't.

Sources and labels for every figure are in the full plan and evidence ledger. Some market figures could not be independently verified during the research window and are flagged there.

THE IDEA

A premium, doctor-led home for peptide health.

Three connected parts: a library people trust, care from independent clinicians, and a small store for things that don't need a prescription. The brand's promise is transparency: we show what's approved, what's compounded, what's still unproven, and what we refuse to sell.



Learn

A plain-language library. Every peptide gets an FDA-status badge and an honest evidence grade — including the ones we don't sell.



Get care

A 3-minute eligibility check, then an independent licensed clinician decides. Licensed US pharmacies fill it. No open cart for prescriptions.



Shop

A separate store for non-prescription products that meet our standards. Nothing 'research-use-only', ever.

How it works for a member



What we will never do



Sell 'research-use-only' peptides

The FDA now treats those websites as unapproved-drug sellers. Several owners have been prosecuted in 2025-26.



Promise results

No before-and-after photos, no miracle claims, no 'same as Ozempic'. Honest evidence grades instead.



Hide fees or trap people in subscriptions

All-in pricing before anyone starts, and cancellation in two clicks. This is also what the FTC is suing over right now.

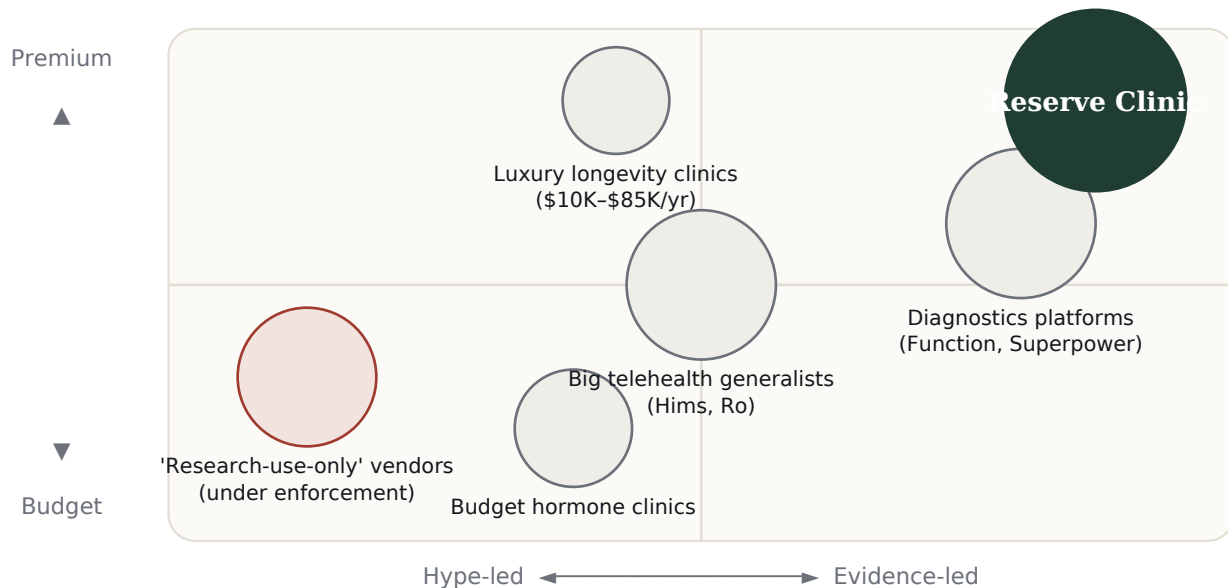


Let marketing touch medical decisions

Clinicians are independent and never paid by the prescription. That protects members, and it protects you.

THE LANDSCAPE

Where Reserve Clinic sits, and what celebrity brands teach us.



Analyst view based on the 75-company audit. Bubble size is rough scale; position is positioning, not revenue.

Celebrity-founded health brands: what actually worked



Owner + real product = it works

Hailey Bieber's Rhode launched with a peptide lip treatment as its hero and sold for \$1B in 2025. Podcast hosts who owned equity in AG1 and Onnit built decade-long businesses.



A rented face doesn't convert

Ladder (LeBron James and friends) reached ~\$4M in sales with 180M+ combined followers. SKKN by Kim folded in 2025 despite the largest audience in our database.



Health trust breaks hard

Goop paid \$145K over unproven claims; Kim Kardashian paid \$1.26M for an undisclosed promotion; Lemme's 'GLP-1 Daily' drew class actions within five months of launch.



The face can't be the whole brand

TB12 lost its founder and had to rebrand. WeightWatchers' stock dropped ~25% in a day when Oprah left. Doctors and standards have to carry equal weight.

59 celebrity relationships were classified from company filings and press. Details and sources are in the full database.

YOUR PROPOSED ROLE

Creative direction and reach, with the medicine kept separate.

This is a sketch for discussion, not an offer. The structure is designed so your name adds awareness and taste while independent doctors and published standards carry the trust.



What you'd shape

Brand vision and creative direction. Founder-led content and the launch moment. Partnerships and introductions. A seat in customer insight sessions.



What stays out of your lane

Which peptides are offered, who qualifies, and what the site claims medically. That belongs to the medical director and compliance, by design and by law.



How it could be structured

Cash retainer, equity that vests with deliverables, a capped royalty, or a mix. We modeled all of them. No terms have been proposed or discussed.



Time and obligations

A defined number of content days per quarter, launch-week availability, and approved scripts for anything that touches health. Everything disclosed as a paid or ownership relationship, every time.

The honest part

- **Disclosure has to be perfect.** The FTC's 2023 rules treat ownership and payment as things every post must disclose. Your name appears on the FTC's 2017 influencer warning-letter list; that history means reporters and regulators will look closely, so the program has to be spotless from day one.
- **Your GLP-1 history is a question, not a problem.** You've been open about Mounjaro. A peptide brand can work with that honesty. What we need to check first is whether any past GLP-1 promotion carries exclusivity that conflicts with this category.
- **Both sides get protection.** Mutual morality clauses, clear termination terms, and rules about how the brand can use your name after any exit.

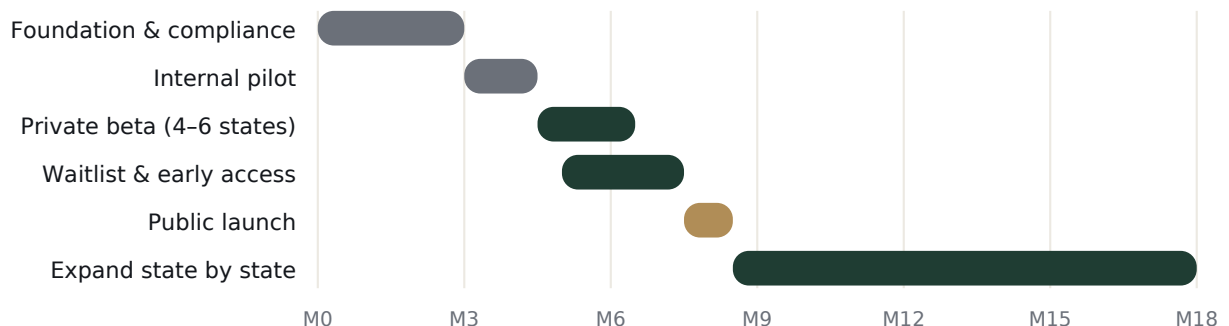
The medical cofounder

A physician medical director would own the formulary, the protocols and every health claim, employed by the clinical entity rather than the brand, with pay that never depends on prescriptions. The doctor proposed for this role has not yet been verified; identity, licenses and board status must be confirmed before anyone is named publicly.

THE PLAN

Build quietly, prove it works, then switch on the reach.

No single big-bang launch. We build the library, sign the clinical and pharmacy partners, run a private beta in a handful of states, grow a waitlist, and only then go public. Your audience is used only after contracts are signed, and it lands on a product that already works.



~9 mo

to public launch

\$4.8M

capital needed, base case
(assumption)

Month 18 \$42.8M

break-even, base case
(assumption)

year-3 revenue, base case
(assumption)

These are modeling assumptions, not forecasts. The conservative case needs about \$12.9M and does not break even within three years; the full model shows every input.

What we'd want to decide with you

- Whether you want to explore this at all, and in what shape: brand partner, cofounder, or advisor.
- Who represents you, and whether any existing deal limits a health or GLP-1 category partnership.
- How much of the brand should be built around you versus around the doctors and standards. Our recommendation: the medicine leads, you amplify.
- Comfort with the disclosure and morality terms that a health brand requires.

What happens next if you're interested

- A conversation with your representatives about scope and category exclusivity.
- In parallel: verify the medical cofounder, engage healthcare counsel, clear the trademark, and build a working prototype of the library and eligibility flow so you can see and shape it before deciding.

Prepared 2026-09-02 from a 43-page plan, a 36-month financial model and an evidence ledger of 790 sources. Some research could not be directly verified under network limits; those gaps are listed in the full plan.